February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy, National Coordinator for Health Information Technology
Attention: HHS Health Sector AI RFI
Mary E. Switzer Building
330 C Street SW
Washington, DC 20201

Dear Assistant Secretary Keane,

The Healthcare Business Management Association (HBMA) is pleased to submit this comment letter in response to your request for information (RFI) titled: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13).

We appreciate ASTP/ONC's focus on understanding how AI can enhance clinical care. We urge ASTP/ONC to also consider how AI's integration into clinical care intersects with the back-end revenue cycle management (RCM) process, which will be an essential factor in the proliferation of AI into the clinical setting. We encourage the agency to issue similar RFIs in the future that focus on the RCM process and to engage with stakeholders like HBMA through meetings and industry focus groups.

Below, please find our responses to specific questions from the RFI.

Question 2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

AI has significant potential to improve the healthcare system. Not only can it help improve clinical outcomes, it can also be used to improve administrative processes that create avoidable burdens on clinicians. These burdens consume time and resources that could otherwise be used on patient care.

Any conversation about the clinical applications of AI must be paired with the administrative functions that accompany the clinical care such as the revenue cycle management (RCM) process, documentation requirements, and quality improvement programs. Clinicians must be adequately reimbursed for the care they provide. As with all medical services, clinicians must be fairly compensated for new clinical applications that utilize AI.

AI should not be a justification to reduce reimbursement rates. The 2026 Medicare Physician Fee Schedule (PFS) controversially finalized an "efficiency adjustment" which reduces Medicare reimbursements due to the assumption that care becomes more efficient as clinicians gain more experience. While this is often true, these efficiencies are more than offset by administrative and financial burdens from government regulations and actions by commercial payers.

Clinicians are more likely to adopt AI tools in clinical care if it can generate efficiencies, decrease burden, improve patient care, and if it is fairly reimbursed. HHS should understand that every health system, hospital, physician office, medical specialty and clinician have different needs and uses for AI in clinical care and the administrative functions of the healthcare system. These needs widely vary both across the entire healthcare system and within individual health systems and practices. Any federal AI policy must be responsive to these individual needs.

HBMA supports the development and use of AI for these beneficial purposes. However, our industry is reliant on private sector innovation to develop and maintain these technologies. Additionally, reimbursement policies set by commercial and government payers will have a major impact on adoption of AI tools.

In addition to reimbursement rates, it is important to understand if payers will establish burdensome documentation and administrative requirements for items and services that incorporate AI or decrease payment rates based on potential efficiency improvements. For example, will clinicians be required to document use of AI tools? Will they need to identify the specific tool that was used? Will payers establish separate payment rates for the same procedure depending on if AI is or is not used?

We encourage HHS to consider the administrative impacts of AI as part of any effort to promote AI in clinical care. HHS should issue a new RFI on how the adoption of AI in clinical care will impact administrative and revenue cycle processes.

We are already seeing commercial payers deploy AI to inappropriately deny higher volumes of claims and prior authorization requests. These companies, which are among the largest corporations in the country, have vast resources to utilize these tools for these purposes. Some of these actions are subject to class action lawsuits and regulatory scrutiny.

We believe that government policies must ensure an even playing field for using AI across all parts of the healthcare system. Frustratingly, commercial payers are increasingly relying on AI to adjudicate claims at the same time they are stifling how clinicians use AI. For example, many commercial payers will communicate with providers using automated AI phone calls. However, these payers refuse to accept calls from providers using the same automated AI phone call technologies. These phone calls are a practical way that AI can reduce administrative burdens by facilitating tedious, time-intensive processes that allows humans to focus their time and energy on more important work.

Question 3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (*e.g.*, relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Liability protections must be a central aspect of any policy that incentivizes the use of AI in clinical care. Clinicians must have a clear understanding of their potential legal liability due to alleged issues caused by AI tools. A robust AI policy must establish a definitive set of legal liability protections for how clinicians use AI in clinical care. While there are many ways to approach creating such protections, the most important part is that legal liability can be clearly understood by all parties. A lack of clarity on this important topic could dissuade clinicians from implementing AI tools in clinical care.

HHS also has a part to play in defining the privacy and security requirements around AI applications for all involved parties, including AI developers, health systems, clinicians and payers. Early and ongoing interpretation of existing laws and regulations and continued updates as needed will help to remove roadblocks and clarify guardrails for implementation and monitoring of AI in clinical settings.

Additionally, AI tools should be structured to enhance and support a clinician's abilities. It should not be used to replace a clinician. For example, some AI tools are designed to provide mental health care but have actually offered incredibly dangerous medical advice. HHS should draw a clear line that prevents AI tools from replacing clinicians.

Question 5. How can HHS best support private sector activities (*e.g.*, accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HBMA believes there must be a process for validating AI tools in a way that differentiates useful AI tools from inadequate or flawed products. We caution HHS (or any federal agency) against declaring itself as an authority on accrediting, approving or certifying AI tools for use in clinical care.

Many organizations are seeking to assert themselves as an AI authority. HHS partners with outside organizations to implement policies such as selecting CAQH CORE to develop operating rules for HIPAA transactions and relying on contractors for quality measure development. We caution against relying on outside vendors to serve as AI certification entities. Past experience with how EHRs were certified by both government and non-government entities despite many flaws with these products suggests a more cautious and deliberate approach is needed. HHS should not rely on an organization to serve as an AI accreditor or certifier until HHS first establishes a clear policy for how AI should be used in clinical care.

At this time, we feel the best role for government is to establish minimum standards as a guidance to industry on which AI tools are appropriate for clinical care. Clinicians and RCM companies want to be compliant. However, this requires clear, concise guidance for what regulators expect and resources to assist with compliance. For example, guidance should clarify what data elements are acceptable for use in large language models (LLMs), or requirements for data storage and retention. Additionally, guidance must be well organized and indexed in a standardized fashion so that they are easily searchable and clearly display updates.

Additionally, HHS should leverage the Medicare reimbursement system to decide which tools it will or will not cover or reimburse. By choosing to reimburse for an item or service that utilizes AI, HHS can control which tools are used in clinical care. We caution HHS against requiring clinicians to document use of AI as that would add more layers to an already burdensome process.

AI is already helping to revolutionize the healthcare RCM industry. The RCM industry is filled with avoidable administrative burdens – barriers that are largely constructed by payers. Processes such as prior authorizations, supplemental documentation requests, denials and appeals, and lack of interoperability make the back-office functions of our healthcare system are more difficult than they need to be. The role of the RCM industry is to absorb these challenges on behalf of medical practices so that clinicians can focus their time and resources on caring for patients instead of administrative requirements.

The RCM industry is now incorporating automation and certain types of AI into our workflows by necessity as a response to these challenges.

Clinician burnout is one of the top health policy priorities for both Congress and regulators. We believe that HHS should approach AI from the perspective of reducing burdens to ease burnout. Many clinicians are choosing to leave the clinical workforce over these burdens. AI tools have the potential to either increase or decrease these burdens. We believe government guidance and regulations must maintain a focus on reducing burnout and must be responsive to concerns from clinicians about how AI could potentially increase burdens.

Question 7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Revenue cycle is a major factor in decisions to adopt AI tools. Clinicians must be fairly reimbursed for using AI tools in clinical care. The RCM industry is responsible for ensuring clinicians are reimbursed for the services they provide. This often means navigating complex billing and coding policies, understanding coverage policies unique to each payer, facilitating documentation requests and managing denials and coding adjustments.

We believe HHS should build on this RFI by shifting its focus from the clinical factors to the administrative and revenue cycle factors related to AI adoption. We suggest HHS issues additional RFIs on these topics. We also recommend that HHS consults with stakeholder organizations such as HBMA to discuss these issues in more detail. HHS should also consider convening focus groups of stakeholder organizations to provide input on HHS' AI policies.

HHS must maintain separation between adoption of AI and quality programs such as MIPS. HHS should not hold clinicians financially accountable for using or not using AI. While AI certainly has potential to improve care quality and efficiency – both of which are the primary goals for HHS value-based payment programs – it is essential that AI adoption remains separated from these programs in these early days of AI development. We support policies that incentivize using AI under conditions described throughout this response. However, we believe such policies should not be included in value-based payment programs, which already create significant financial and administrative burdens for clinicians with minimal potential rewards to return the investment necessary to successfully participate in these programs. .

Question 8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

HBMA supports interoperability but is frustrated that it still does not exist. While improvements have been made, many EHR systems are still not interoperable. In some cases, products are not interoperable with other products from the same vendor. New policies that leverage standard APIs have promise to greatly improve interoperability. However, widespread API adoption will be needed to achieve these goals. We believe AI could help address this issue by serving as an intermediary to ensure that health data is interoperable across EHR platforms.

Importantly, these same interoperability concerns apply to AI products used in clinical care. AI has great potential to connect products, clinicians and other technology into a cohesive care delivery platform. We encourage HHS to emphasize interoperability in any policy that promotes the use of AI tools in clinical care.

Question 9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

From the perspective of the RCM company, patients are frustrated with many aspects of the healthcare system. These "pain points" include confusion over medical bills, misunderstanding of their plan's benefits and provider network, frustration over health insurance coverage decisions such as preauthorization care delays and coverage denials, and lack of price transparency, among other things.

We believe AI can improve the consumer experience for patients by addressing many of these issues. For example, AI tools can help patients better understand their health insurance and coverage decisions, as well as make price transparency data easier to provide and interpret.

However, patients typically prefer to interact with a human rather than a bot. Perhaps agentic AI will improve to a point where this dynamic changes. However, that is not the current reality. It is therefore important to provide transparency to patients if they are dealing with an AI or bot instead of a human.

Studies show patients are skeptical of how is used in clinical care. Patients continue to strongly trust their physicians. Clinicians must be careful to balance AI's potential to improve clinical care with patient's mistrust of these tools.

Providing transparency around how AI tools are developed, tested, and protected against bias are essential to building trust in these tools among both patients and clinicians. We are concerned that AI models could be trained on flawed data sources. For example, many EHRs contain outdated, redundant and inaccurate information. AI tools that are trained on EHR data could result in flawed end-use products.

Care quality and safety must always be a higher priority than any potential efficiencies gained by AI tools. Further, AI cannot and should not ever replace a clinician. There are many interesting use cases in specialties such as radiology and mental health. However, AI tools are intended to support the clinician, not replace them.

With high demand for health care and shortages of clinicians in many areas, AI can make care more efficient both clinically and administratively.

Conclusion

Thank you for your consideration of our comments, especially those that urge ASTP/ONC to consider how the RCM industry will play an important role regarding how AI is incorporated into clinical care. Please do not hesitate to contact HBMA Director of Government Relations Matt Reiter (reiterm@capitolassociates.com) or HBMA Executive Director Brad Lund (brad@hbma.org) if you wish to discuss our recommendations in more detail.

Sincerely,

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President, HBMA